



# Fiona Stanley Fremantle Hospitals Group

Procedure

## Waitlist Management: Kara Maar

Reference #: FSFH-MENH-PRO-0016

### Scope

| Site               | Service/Department/Unit | Disciplines                                         |
|--------------------|-------------------------|-----------------------------------------------------|
| Fremantle Hospital | Kara Maar               | Medical, Nursing, Allied Health, and Clerical Staff |

### 1. Introduction

This document clarifies the waitlist process when a referred consumer is suitable for an episode of care with Kara Maar however the service does not have capacity to immediately allocate a Care Coordinator and offer assessment and intervention.

### 2. Terminology

|                                                         |                                                                                                                                                                                                                                                                                                                       |
|---------------------------------------------------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Kara Maar                                               | Specialist Community Eating Disorder Service for South Metropolitan Health Service (SMHS). Kara Maar is a community based mental health service providing assessment, care coordination, clinical monitoring, and multidisciplinary therapeutic interventions to people aged 16 years and over in the SMHS catchment. |
| Care Coordinator                                        | The nominated person within the multidisciplinary team (MDT) who has the responsibility for coordinating, facilitating, and integrating mental health treatment, care and support to a mental health consumer, their family and significant others.                                                                   |
| Psychiatric Services Online Information System (PSOLIS) | Database for entering information related to the provision of services to persons from Mental Health Services.                                                                                                                                                                                                        |
| Triage Clinician                                        | A clinician allocated to receive, determine eligibility, process, and prioritise referrals to the service and provide general information and advice. To coordinate the waitlist monitoring and management process.                                                                                                   |
| Multi-disciplinary Team (MDT)                           | A team of clinicians from a range of disciplines who work together to deliver comprehensive care that addresses as many consumer needs as possible.                                                                                                                                                                   |

**Intake Meeting**

Meeting attended by the multidisciplinary team to facilitate discussion of new referrals and identified consumers of concern and determine actions to be taken.

### 3. Procedure

- The Intake Meeting occurs twice weekly and is attended by the MDT.
- New referrals are presented for discussion by the Triage clinician and a team decision is made regarding consumer suitability for service and referral acceptance.
- When clinician caseloads are at capacity and the consumer cannot be allocated to a Care Coordinator at time of referral acceptance, the consumer will be waitlisted and the plan for monitoring and management will be determined by the MDT.
- The waitlist will be prioritised by the following:
  - High priority - rapid deterioration of medical status and/or escalation of eating disorder behaviours, unable to access private services and currently not engaged with any other public or private services, to be allocated upon first availability.
  - Medium priority - slow deterioration of medical status and/or escalation of eating disorder behaviours, but may be engaged with public or private services, to be allocated when there are no high priority referrals to be allocated.
  - Low priority - medical status and eating disorder behaviours are stable, access to private services is an option to consider, is engaged with public or private services, last to be allocated.

#### 3.1. The Triage clinician or delegated mental health clinician will take responsibility for:

- Contacting the consumer and if appropriate, family/significant others to inform of acceptance onto the service waitlist and to provide the following information:
  - Kara Maar contact phone number,
  - 24-hour mental health crisis and emergency services numbers should there be a deterioration in mental or physical health status or if there are concerns about risk to self or others,
  - Kara Maar Waitlist Information Pack via email.
- Gaining verbal consent from consumer to liaise with existing supports including health care team, mental health services, family, and significant others, as required, to develop a shared understanding of each stakeholder's

role whilst the consumer is on the waitlist. Where a consumer is < 18 years and not a mature minor, this information will also be provided to the legal guardian and documented in the medical record.

- The Triage clinician will request GP monitoring as clinically indicated and results forwarded to Kara Maar with reinforcement of escalation to acute services if deterioration occurs and to advise Kara Maar accordingly.
- Informing the referrer by phone and/or email the consumer is accepted to Kara Maar and is currently waitlisted.
- Requesting the referrer (if appropriate) maintain contact with the consumer.
- The Triage clinician or delegated mental health clinician should maintain at a minimum fortnightly contact with the consumer, in the context of consumer acuity and level of appropriate existing support to:
  - Seek feedback as appropriate on any change in physical, mental health and/or eating disorder behavioural status since last contact.
  - Encourage continued willingness and motivation to engage with the service.
  - Provide empathic listening and remind the consumer that they can contact the service or existing/appropriate support should their physical or mental health, or social circumstances change.
  - Advise the importance of GP monitoring as clinically indicated and encourage engagement with the GP.
- Updating PSOLIS to reflect the change in the consumer's status from "in progress" to "waitlist" and completing documentation in the consumer's medical record.
- In the event direct contact with the waitlisted consumer has been unsuccessful, the Triage clinician will inform the Kara Maar multidisciplinary team at the next intake meeting, the referrer and if appropriate family/significant others. A plan will be formulated by the MDT and if no immediate actions required a letter will be sent to the consumer requesting contact with Kara Maar within two weeks.
- Documentation of each contact in the consumer's medical record and any additional clinical information.

### **3.2. The Triage clinician or delegated mental health clinician will ensure clinical escalation via:**

- Utilisation of the priority system to identify any escalating risks:
  - discuss with the MDT and take agreed action.

- In the event of serious and imminent risk the Triage clinician will take appropriate action in collaboration with MDT and family/significant others, as appropriate, and advise the referrer or teams involved:
  - the use of crisis and emergency service pathways may be considered and a risk assessment to be completed.
- Clinical incidents related to consumers on the Kara Maar Waitlist will be notified in Datix Clinical Incident Management System

## 4. Compliance monitoring

- The Kara Maar Team Leader will monitor compliance with this procedure via clinical documentation audits at site and routine clinical incident monitoring.

## 5. Related Policy Documents

- WA Health:
  - [Statewide Standardised Clinical Documentation for Mental Health Services MP 0155/21](#)
  - [Community Mental Health Status Assessments: Role of Mental Health Clinicians MP 0099/18](#)
  - [Clinical Incident Management policy MP 0122/19](#)
- SMHS:
  - [Acute Response to mental health emergencies/crises SMHS: 17](#)
  - [Care Coordination in Mental Health SMHS: 13](#)
- FSFHG:
  - Clinical Handover FSFH-HW-POL-0035
  - Clinical Documentation FSFH-HW-POL0039
  - Clinical Risk Assessment and Management FSFH-HW-POL-0017

## 6. Related Standards

- NSQHS Standards:
  - Clinical Governance
  - Partnering with consumers
  - Communicating for Safety
  - Recognising and Responding to Acute Deterioration

- Chief Psychiatrists Standards for Clinical Care:
  - Assessment
  - Consumer and carer involvement in individual care
  - Risk assessment and management

## 7. Authorisation

| EXECUTIVE SPONSOR: Service Director, Service 5 |             |                       |                                    |                  |              |
|------------------------------------------------|-------------|-----------------------|------------------------------------|------------------|--------------|
| Version                                        | Date Issued | Compiled/Revised By   | Committee/Consumer Group Consulted | Endorsed By      | Revision due |
| 1                                              | 09/2024     | Team Leader Kara Maar | S5 SQR Committee                   | S5 SQR Committee | 10/2026      |